

Week 8 — Diagnosis by Inquiry (cont.) & Palpation

EAM310 · EAST ASIAN MEDICINE DIAGNOSIS I · VUIM SUMMER 2026

⚠ Quiz 5 — September 4, 2026

Class: Friday September 4, 2026 · 9AM–1PM

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REQUIRED READINGS — DUE BEFORE SEPTEMBER 4

FCM CHAPTER 24 — INQUIRY (REVIEW) — MACIOCIA

- Continued application of the 16 Questions
- Refining the full intake procedure with live cases

FCM CHAPTER 25 — PALPATION — MACIOCIA

- Palpating skin: temperature, moisture, texture
- Palpating limbs, chest, abdomen
- Diagnostic points: Back-Shu, Front-Mu, Lower He-Sea
- CAM Ch.9 — Palpation (切診) as fourth of Four Methods

WEEK 8 LEARNING OBJECTIVES

- Continue applying the 16 Questions to live patient interviews
- Differentiate Full-Heat vs Empty-Heat by dorsum/palm
- Palpate the apical beat (Xu Li) and interpret findings
- Identify Back-Shu, Front-Mu, Lower He-Sea point locations
- Interpret skin temperature at graded pressure depths
- Apply the cold-hands-and-feet differential (6+ organ patterns)
- Interpret chest, xiphoid-area, and abdominal palpation findings
- Integrate palpation findings with prior inquiry/observation data

ASSESSMENT CALENDAR

Date	Event	Material Covered
Aug 28	Week 7 Class	Diagnosis by Inquiry — 16 Questions
Sep 4	⚠ Quiz 5	Diagnosis by Inquiry & Palpation — skin, limbs, chest, abdomen, points
Sep 11	Quiz 6	Week 9 material — Palpation cont. + Hearing/Smelling
Sep 18	FINAL + Journal 26–50	Weeks 6–9 comprehensive

IN-CLASS PRACTICE — WHAT TO EXPECT

FULL-BODY PALPATION LAB

Week 8 includes hands-on partner palpation practice. Come ready to:

- Compare dorsum vs palm temperature on a partner's hand (Full-Heat vs Empty-Heat)
- Palpate the apical beat (Xu Li) and describe strength/quality
- Practice abdominal palpation — distinguish moving vs fixed masses
- Locate and palpate at least 2 Back-Shu and 2 Front-Mu points, noting tenderness

SKIN TEMPERATURE — COLD PATTERNS

General cold feeling	Cold pattern
Four limbs cold	Spleen- and/or Stomach-Yang deficiency
Loins, lower back, or feet cold	Kidney-Yang deficiency
Lower abdomen cold	Spleen-Yang deficiency

SKIN TEMPERATURE — HEAT PATTERNS (PRESSURE-DEPENDENT DIAGNOSIS)

Skin actually feels hot to touch	Damp-Heat
Hot on FIRST touch, fades with sustained pressure	Exterior Wind-Heat — pathogen still on Exterior
Hot on MEDIUM pressure, NOT on heavy pressure	Interior Heat in Middle Burner or Heart
Hot on HEAVY pressure, nearly to the bone	Empty-Heat from Yin deficiency

Key diagnostic principle: The DEPTH at which heat is felt tells you the depth of the pathology. Superficial heat that fades = exterior. Heat that only appears deep = the deepest Yin-deficiency Empty-Heat.

SKIN MOISTURE & TEXTURE

Moist skin	Invasion of exterior Wind (Cold or, more usually, Heat)
Moist, NO exterior symptoms	Spontaneous sweating from Lung-Qi deficiency
Dry skin	Blood deficiency or Lung-Yin deficiency
Rough skin	Painful Obstruction Syndrome from Wind
Scaly and dry skin	Exhaustion of Body Fluids, or severe Liver-Blood deficiency/dryness
Swollen, PITTING on pressure	"Water swelling" — Spleen- and/or Kidney-Yang deficiency
Swollen, NOT pitting (no pit left)	"Qi swelling" — Dampness or Qi stagnation

COLD HANDS & FEET — FULL DIFFERENTIAL

Pattern	Distinguishing Feature
COMMON CAUSES	
Cold hands AND feet	Spleen- or Stomach-Yang deficiency
Cold hands ONLY	Lung- and/or Heart-Yang deficiency
Cold feet ONLY	Kidney-Yang deficiency
Cold hands and feet in WOMEN	Blood deficiency
Cold hands in women specifically	Heart-Blood deficiency
Cold feet in women specifically	Liver-Blood deficiency
Cold FINGERS AND TOES specifically	Liver-Qi stagnation (Qi can't reach extremities)

LESS COMMON CAUSES

Cold hands/feet + contradictory hot symptoms	Phlegm or interior Heat obstructing Qi circulation
Cold hands + intense Heat signs elsewhere	Heat in Pericardium at Nutritive Qi level (Four Levels) — Heat trapped, obstructing circulation to extremities

DORSUM VS PALM — FULL-HEAT VS EMPTY-HEAT

Hot DORSUM (back) of hand	Full-Heat
Hot PALM of hand	Empty-Heat

The 發熱 fā rè distinction: Classical "fever" in exterior Wind invasion actually refers to the objective hot feeling of the dorsum of hands/forehead on palpation — NOT necessarily a measured fever. Spiritual Axis Ch.74: hot palm = Heat in Intestines; cold palm = Cold in Intestines.

PALPATION OF HANDS & FEET — TEMPERATURE DIAGNOSIS	
Lung- and/or Heart-Yang deficiency	Coldness of hands ONLY
Spleen- and/or Kidney-Yang deficiency	Coldness particularly of feet, but often hands too — WHOLE LIMBS cold
Stomach-Yang deficiency	Coldness of both hands and feet
Heart-Blood deficiency (women)	Cold hands
Liver-Blood deficiency (women)	Cold feet
Liver-Qi stagnation	Cold hands/feet, particularly fingers and toes — NOT a Yang deficiency, it's stagnant Qi unable to reach extremities

Less common causes: Phlegm in the Interior obstructing Qi circulation to limbs (may also occur with Phlegm-Heat, producing contradictory hot/cold symptoms) · Very pronounced interior Heat obstructing Qi circulation so it cannot warm hands/feet — produces contradictory hot/cold symptoms (e.g., Heat in Pericardium at Nutritive Qi level: Dark-Red tongue no coating, night fever, mental restlessness — *but cold hands*).

PALPATING THE HAND — ORGAN CORRESPONDENCE MAP

The diagram illustrates the correspondence between specific zones on the palm and various internal organs. The left hand (palm side) is divided into six numbered zones: 1 (Chest Cavity), 2 (Respiratory), 3 (Abdominal), 4 (Abdominal), 5 (Respiratory), and 6 (Intestine/Rectum). The right hand (back side) is divided into zones corresponding to specific organs: Vagina/Cervix, Bladder, Penis, Prostate, Kidney, Uterus, Spleen, Appendix, Liver, Pancreas, Colon, Small intestine, Lung, Lungs, Heart, Stomach, and Bronchi. The diagram is credited to Maciocia 2015.

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Figure 25.6 — Organ positions in hand diagnosis (Maciocia 2015)

The palm reflects internal organs in a pattern similar to the ear — visualize a small baby superimposed on the palm. Diagnosis: gently press the relevant area. **Sharp pain** = Full condition. **Dull soreness** = Empty condition of that organ.

Hand Zone	Corresponds To
Chest cavity organs	Lungs, Heart, Bronchi
Upper abdominal cavity	Liver, Spleen, Stomach, Pancreas
Lower abdominal cavity	Small intestine, Colon, Appendix
Reproductive/urinary organs	Kidney, Bladder, Uterus, Prostate, Cervix, Penis, Vagina
Respiratory organs	Lungs, Bronchi
Intestines/rectum area	Colon, Rectum

APICAL BEAT — "INTERIOR EMPTINESS" (XU LI 虚里)

- ❖ Refers to the pulse of the heart of the left ventricle, which can be felt in the fifth intercostal space
- ❖ Described in Su Wen 18
- ❖ Clearly felt, not hard, relatively slow: Normal
- ❖ Feeble and without strength: deficiency of the Gathering Qi Zong Qi
- ❖ Strong and hard: excess condition of the Lungs and/or Heart
- ❖ Large but empty: Heart-Qi deficiency
- ❖ Stopping and starting: severe shock or alcoholism
- ❖ Discrepancy between apical and radial pulse: poor prognosis
- ❖ Rapid: shock, fright or outburst of anger

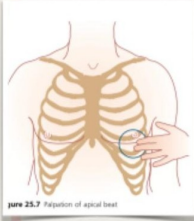


Figure 25.7 Palpation of apical beat (Maciocia 2015)

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-Maciocia 2015

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Figure 25.7 — Palpation of apical beat (Maciocia 2015)

Palpate over the left ventricle apex. Traditionally considered the end of the Stomach Great Connective Channel and reflective of the Gathering Qi (Zong Qi) of the chest.

Finding	Meaning
Clear, not hard, relatively slow	Normal
Feeble, without strength	Deficiency of Gathering Qi (Zong Qi)
Strong and hard	Excess condition of Lungs and/or Heart
Large but empty	Heart-Qi deficiency
Stopping and starting	Severe shock or alcoholism
Discrepancy between apical and radial pulse	Poor prognosis
Rapid	Shock, fright, or outburst of anger
Pulsation cannot be felt at all	Phlegm or hiatus hernia

AREA BELOW THE XIPHOID PROCESS

- ❖ Reflects the state of the Upper Burner Qi (Lung and Heart Qi and Zong Qi/ Gathering Qi)
- ❖ Should feel soft relative to the rest of the abdomen
- ❖ Hard on palpation: Full condition, Qi stagnation (from emotional problems)
- ❖ Too soft on palpation: Heart deficiency

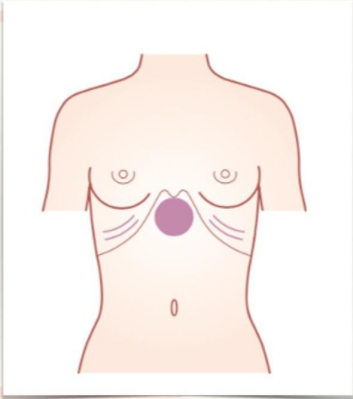


Figure 25.8 Area below the xiphoid process (Maciocia 2015)

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Figure 25.8 — Area below the xiphoid process (Maciocia 2015)

Should feel relatively SOFTER than the rest of the abdomen — this reflects Upper Burner Qi (Lung, Heart, Gathering Qi). Compare directly to surrounding abdomen.

Relatively soft (as expected)	Normal — smooth flow of Lung/Heart-Qi
Hard on palpation	Full condition, Qi stagnation from emotional problems — constriction of Corporeal Soul
Too soft on palpation	Heart deficiency

PALPATING THE CHEST — CENTER VS SURROUNDING

Very tender, light palpation, center (Ren-17 Shanzhong)	Heart-Blood stasis
Tender around the center	Excess condition of Lungs — often Phlegm retention
Palpation RELIEVES discomfort	Deficiency condition of Heart or Lungs
Light palpation relieves, deep pressure elicits discomfort	Combined Deficiency and Excess

PALPATING THE BREAST (LUMPS)

Critical caveat: Chinese medicine breast palpation is NEVER a substitute for Western diagnosis. Never rely on TCM palpation to distinguish benign from malignant — its purpose is identifying the CM pattern only.

Quality	Indicates
Relatively soft	Phlegm
Relatively hard	Blood stasis
Distinct edges	Phlegm
Indistinct edges	Toxic Heat
Mobile on palpation	Phlegm
Immovable on palpation	Blood stasis or Toxic Heat

Clinical Picture	Likely Diagnosis (CM pattern)
Small, movable, distinct edges, size varies w/ cycle	Fibrocystic disease — Phlegm + Qi stagnation
Single, hard, movable, distinct edges, mild pain	Fibroadenoma — Phlegm + Blood stasis
Single, hard, IMMOVABLE, indistinct margins, painless	⚠ Possible carcinoma — Phlegm + Qi stag + Blood stasis, Penetrating/Directing Vessel disharmony

ABDOMINAL PALPATION — QUICK REFERENCE

❖ Discussed in the *Nan Jing* and *Shang Han Lun*

❖ Practitioners palpate various areas on the abdomen

❖ By discerning the sensitivity and palpations of these locations they then make inferences about the balance of the various Organs

❖ Japanese physicians would further evolution and methods of diagnosis.



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Abdominal palpation technique (Schrier Lecture Slides)

Finding	Indicates
Solid, resilient, elastic	Good Original Qi — NORMAL
Hard / painful on palpation	Full condition
Too soft / ache relieved by palpation	Empty condition
Masses that MOVE under fingers	Qi stagnation
Masses FIXED and hard	Blood stasis

Core palpation principle (applies everywhere on the body): Moving mass = Qi. Fixed mass = Blood. This single rule underlies chest, abdomen, and channel-point palpation alike.

DIAGNOSTIC POINT CATEGORIES 診斷穴 — OVERVIEW

Point Category	Function
Back-Shu (Transporting) Points	Where Qi/Blood of a specific organ "infuses" on the back — full table on next page
Front-Mu (Collecting) Points	Particularly reactive to pathological internal-organ changes — full table on next page
Lower He-Sea Points	Used for diagnosis of the 6 Fu (Yang) organs specifically — full table on next page
Ah Shi Points	"Oh yes!" points — tender spots with no fixed location, found by palpation alone
Sharp pain (spontaneous or on pressure)	Full condition of the relevant organ
Dull soreness (spontaneous or on pressure)	Empty condition of the relevant organ

NAN JING 16/56 — FIVE-PHASE ABDOMINAL PALPATION MAP

- Below the sternum reflects
 - Heart/Fire (Ren 12-15)
- Around the umbilicus reflects
 - Spleen/Earth (Ren 7-Ren 12)
- Below the umbilicus reflects the
 - Kidney/Water (Ren 7-Pubis)
- Right side below the rib cage reflects
 - Lung/Metal (Right Abdominal Region)
- Left side below the rib cage reflects
 - Liver/Wood (Left Abdominal Region)

Matsumoto K, Birch, S (1988) Hara Diagnosis: Reflections of the Sea
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5-Phase Abdominal Palpation Map (Matsumoto & Birch 1988, Hara Diagnosis: Reflections of the Sea)

Below the sternum	Heart / Fire (Ren-12 to Ren-15)
Around the umbilicus	Spleen / Earth (Ren-7 to Ren-12)
Below the umbilicus	Kidney / Water (Ren-7 to Pubis)
Right side below rib cage	Lung / Metal (Right Abdominal Region)
Left side below rib cage	Liver / Wood (Left Abdominal Region)

JAPANESE ABDOMINAL DIAGNOSIS — FUKU SHIN 腹診

- Waichi Sugiyama (1610-1694)
 - Credited for the development of acupuncture as we know today in Japan and the further contributions to abdominal diagnosis
- Todo Yoshimasu (1702-1773)
 - Would develop the practice by creating diagnostic indicators gently palpating the abdomen and associating it with a traditional herbal formula
- Bunrei Inaba & Yoshitora (Shukuko) Wakuda
 - further the development of Fuku Shin in the 19th Century

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Fuku Shin abdominal zone map (Schrier Lecture Slides)

Waichi Sugiyama (1610–1694)	Credited for developing acupuncture as practiced in Japan today and contributing to abdominal diagnosis
Todo Yoshimasu (1702–1773)	Developed the practice by gently palpating the abdomen and associating findings with a traditional herbal formula
Bunrei Inaba & Yoshitora (Shukuko) Wakuda	Further developed Fuku Shin in the 19th century

MASTER TUNG PALM DIAGNOSIS 掌診

Master Tung Palm Diagnosis

- ❖ Examine the palm for blood vessels, abnormal spots tenderness etc.
- ❖ From index finger to LU 10 = Lung
- ❖ From middle finger to PC 8 = Heart
- ❖ From ring finger to wrist = Liver & Spleen
- ❖ From pinky to wrist = Kidneys



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Master Tung Palm Diagnosis — annotated hand (Schrier Lecture Slides)

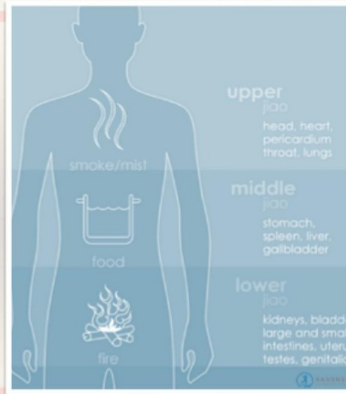
Index finger to LU-10	Lung zone
Middle finger to PC-8	Heart zone
Ring finger to wrist	Liver & Spleen zone
Pinky to wrist	Kidneys zone

Vessel Color	Meaning	Skin Color	Meaning
Light blue	Cold and deficiency	Blue or black	Pain
Dark blue	Pain and excess	Red or yellow	Heat
Red or purple	Heat and inflammation	White	Cold
—	—	Yellow and atrophy	Deficiency

Very dark/gloomy color = pathogen has invaded to interior. Light color = pathogen still on exterior. Palpate palm: soft+atrophy=deficiency; firm+tender=excess; warm=warm abdomen; cold=cold abdomen.

PALPATING THE THREE JIAO 三焦

- ❖ Upper Jiao situated in the chest and lies above the diaphragm.
 - ❖ *Contains the Heart, Pericardium and Lung*
- ❖ Middle Jiao lies between the diaphragm and the navel.
 - ❖ *Contains the Stomach, Spleen, Liver and Gall Bladder, and joins the Lower Burner at the navel*
- ❖ Lower Jiao lies below the navel.
 - ❖ *Contains the Small and Large Intestines, Bladder and Kidney.*



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Upper / Middle / Lower Jiao — organ contents by region (Schrier Lecture Slides)

Upper Jiao	Chest, above diaphragm — Heart, Pericardium, Lung
Middle Jiao	Between diaphragm and navel — Stomach, Spleen, Liver, Gall Bladder; joins Lower Burner at navel
Lower Jiao	Below navel — Small/Large Intestines, Bladder, Kidney

Purpose of Three Jiao palpation: assess warmth/strength of Qi in each Burner · determine if moxibustion/warming should be significant in treatment · assess treatment progress over time · assess patient's reaction to physical contact.

WHAT PALPATION CAN TELL US

Information about various areas of the body, the meridians, and acu-points
Overall condition & general state of meridians and organs

Finding Category

- Stagnations (local or generalized)
- Ah Shi / Trigger Points
- Channel involvement
- Severity of condition
- Organs and ligaments (health, flexibility, location)
- Adhesions/obstructions (stagnations, blockages, dampness)

CHANNEL PALPATION IN THE CLASSICS

Involves tracing the pathways of the 12 primary channels to determine quality of Qi and Blood flow. Discussed in Huangdi Neijing (Su Wen 20, Ling Shu 20/51/73/75), Nan Jing 13, Zhang Zhongjing's Shang Han Lun, Sun Simiao's Qian Jin Yao Fang, and Li Shizhen's Qijing Ba Mai Kao.

3 TECHNIQUES IN PALPATING THE CHANNELS

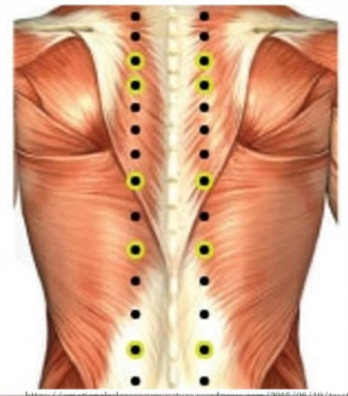
Technique	Method & Interpretation
① Palpation	Running fingers along the course of the channels, often below elbows/knees. Palpate 3 times, pressing a bit harder each pass — some changes are clear at the surface but not deeper, and vice versa. Notice structural changes (muscle tension, nodules, bumpiness).
② Pressing	Use thumb/fingertips to check tenderness along a channel or at specific points. Tenderness/pain = Excess condition. More pain = accumulation of Xie Qi (pathogenic Qi). Weak & soft with/without tenderness = Qi stagnation. Slippery and/or hard = Phlegm.
③ Feeling	Discern noticeable changes in temperature, moisture, skin texture. Compare areas of the body to each other (not necessarily restricted to the channels).

AGGRESSIVE ENERGY (AE) TREATMENT — 5-ELEMENT CONTEXT

Aggressive Energy (AE) Treatment

- ❖ 5 Element Worsley treatment block created by “toxic” qi circulating along the Ke Cycle/Control Cycle between the yin organs
- ❖ Hinted at in *Su Wen* Chapter 65
- ❖ Usually comes from an internal cause ie.. family/relationship problems, financial worries, work issues, or shocks. Though it can come from external causes
- ❖ Points used: BL 13, BL 14, BL 15, BL 18, BL 20, BL 23

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<https://emotionalreleaseacupuncture.wordpress.com/2016/08/19/treatment-5-aggressive-energy-calming-the-organs/>

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AE treatment points on the back (Schrier Lecture Slides)

5-Element Worsley treatment block created by "toxic" Qi circulating along the Ke (Control) Cycle between the Yin organs
Hinted at in *Su Wen* Chapter 65
Usually comes from an internal cause — family/relationship problems, financial worries, work issues, or shocks. Can also come from external causes.
Points used: BL-13, BL-14, BL-15, BL-18, BL-20, BL-23

Tables

Universal rule for all three tables below: Sharp pain (spontaneous or on pressure) = Full condition of that organ. Dull soreness (spontaneous or on pressure) = Empty condition of that organ.

BACK-SHU (BACK-TRANSPORTING) POINTS 背俞穴 — BLADDER CHANNEL

Back Shù俞

- Shù 俞 could be translated as “transport” indicating the qi of the 12 Channels is transported to the surface and is accessible through these points.
- Located on the back the shù 俞 points access particularly strong reserves of qi to provide strong tonification or sedation to the function of their corresponding organ.
- Note: that the shù 俞 points are located relatively over their respective organs

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12 Points

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12 Back-Shu points labeled on the back (Schrier Lecture Slides)

Located on the first lateral line of the back, 1.5 cun lateral to the spine (except BL-14/15 pericardium/heart pairing nuance). Each point is where that organ's Qi and Blood "infuses" — the most direct back-diagnostic point for its organ.

Point	Name	Organ	Vertebral Level
BL-13	Feishu	Lung	T3, 1.5 cun lateral
BL-14	Jueyinshu	Pericardium	T4, 1.5 cun lateral
BL-15	Xinshu	Heart	T5, 1.5 cun lateral
BL-18	Ganshu	Liver	T9, 1.5 cun lateral
BL-19	Danshu	Gallbladder	T10, 1.5 cun lateral
BL-20	Pishu	Spleen	T11, 1.5 cun lateral
BL-21	Weishu	Stomach	T12, 1.5 cun lateral
BL-22	Sanjiaoshu	Triple Burner	L1, 1.5 cun lateral
BL-23	Shenshu	Kidney	L2, 1.5 cun lateral
BL-25	Dachangshu	Large Intestine	L4, 1.5 cun lateral
BL-27	Xiaochangshu	Small Intestine	S1, 1.5 cun lateral
BL-28	Pangguangshu	Bladder	S2, 1.5 cun lateral

ADDITIONAL DIAGNOSTIC BACK POINTS (BEYOND STANDARD BACK-SHU SET)		
BL-43 Gaohuangshu	Reflects the state of the Lungs — often used when LU-Shu (BL-13) is not sufficiently reactive	
BL-53 Zhishi	Often sore in Kidney disease — a secondary Kidney diagnostic point	
BL-31 Shangliao	These four (the "Eight Liao" upper points) reflect the state of the reproductive system, particularly in women	
BL-32 Ciliao		
BL-33 Zhongliao		
BL-34 Xialiao		

FRONT-MU (ALARM / COLLECTING) POINTS 募穴

-
- Alarm (Mu) Points**
- right

LUJ-1
(LU)

GB-24
(GB)

LR-14
(LR)

LR-13
(SP)

GB-25
(KG)

ST-25
(LI)

— · — · —

left

LUJ-1
(LU)

CV-17
(upper jao)

CV-15
(PC)

CV-14
(VT)

GB-24
(GB)

LR-14
(LR)

LR-13
(SP)

GB-25
(KG)

ST-25
(LI)
- CV-12
(ST/upper jao)

— · — · —

CV-7
(lower jao)

CV-5
(TE)

CV-4
(SI)

CV-3
(BL)

— · — · —

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Located on the front/chest/abdomen, on the channel most anatomically close to the organ — NOT necessarily on that organ's own channel. Particularly reactive to acute or Full conditions of their organ.

Point	Name	Organ	Location
LU-1	Zhongfu	Lung	Lateral chest, 1st intercostal space level, 6 cun lateral to midline
REN-17	Shanzhong	Pericardium	Midline, level with 4th intercostal space (nipple line)
REN-14	Juque	Heart	Midline, 6 cun above umbilicus
LIV-14	Qimen	Liver	6th intercostal space, directly below nipple
GB-24	Riyue	Gallbladder	7th intercostal space, below LIV-14
LIV-13	Zhangmen	Spleen	End of 11th rib (free-floating rib tip)
REN-12	Zhongwan	Stomach	Midline, 4 cun above umbilicus
REN-5	Shimen	Triple Burner	Midline, 2 cun below umbilicus
GB-25	Jingmen	Kidney	Free end of 12th rib (lateral)
REN-4	Guanyuan	Small Intestine	Midline, 3 cun below umbilicus
REN-3	Zhongji	Bladder	Midline, 4 cun below umbilicus
ST-25	Tianshu	Large Intestine	2 cun lateral to umbilicus

Unique to the six Fu (Yang) organs — three sit directly on their own channel (ST/GB/BL), three "borrow" a Lower He-Sea point on the Stomach or Bladder channel because their own channel doesn't reach the lower leg with a suitable He-Sea point.

Point	Name	Fu Organ	Note
ST-36	Zusanli	Stomach	On its own channel
GB-34	Yanglingquan	Gallbladder	On its own channel
BL-40	Weizhong	Bladder	On its own channel
ST-37	Shangjuxu	Large Intestine	"Borrowed" — on Stomach channel
ST-39	Xiajuxu	Small Intestine	"Borrowed" — on Stomach channel
BL-39	Weiyang	Triple Burner	"Borrowed" — on Bladder channel

No fixed location, no fixed name — identified purely by palpation finding a tender or reactive spot. Named for the patient's exclamation ("Ah, yes — right there") when the point is pressed. Common in musculoskeletal/Bi Syndrome presentations, and can appear anywhere along a channel or off-channel entirely.

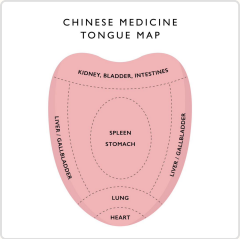
PATTERN NAME	DISEASE CATEGORY	WEEK / QUIZ

ETIOLOGY	ORGAN SYSTEM	DATE

CLINICAL ESSENTIALS — CASE NOTES

PALPATION 切診 · SKIN/CHEST/ABDOMEN	PULSE 脈診 · CUN · GUAN · CHI
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CHINESE MEDICINE TONGUE MAP



TEMP / TEXTURE / MASSES / POINTS

QUALITY / OVERALL

TREATMENT PRINCIPLE 治則

KEY POINTS / HERBS

Jonathan Centeno · D.AcHM Candidate · VUIM Pg _____

溫故知新 · 온고지신

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